Table A 1 Schedule of Events for All Patients up to Month 6

Phase	Screening Phase [a]	Conditioning Phase LDC	Treatment and Primary Follow-up Phase											
Days	≤28 days Before LDC	D -5, -4, -3	D0 [b]	D1	D3	D7	D10	D14	D21	D28/ M1	M2	M3	M4	M6
Visit window				±1 d	±1 d	±1 d	±1 d	±2 d	±2 d	±4 d	±4 d	±14 d	±14 d	±14 d
Informed consent	X													
Inclusion/exclusion criteria	X													
Medical history and patient demographics	X													
ECHO or MUGA	X													
ECOG performance status	X	Day -5 (before LDC)								X	X[c]	X[c]	X[c]	X[c]
12-lead ECG	X	Day -5 (before LDC)												X[c]
Pregnancy tests [d]	X	Day -5 (before LDC)	As clinically indicated											
Height and weight [e]	X	Day -5 (before LDC)								X	X[c]	X		X
Safety assessments														
Physical examination	X	Day -5 (before LDC)	X	X	X	X	X	X	X	X	X[c]	X	X[c]	X
Vital signs and SpO_2	X	D -5 to D -3	X [f]	X	X	X	X	X	X	X	X[c]	X	X[c]	X
Hematology, serum chemistry, coagulation, serum immunoglobulins [g]	X	Day -5 (before LDC)	X [h]	X	X	X	X	X	X	X	X[c]	X[c]	X[c]	X[c]

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Table A 1 Schedule of Events for All Patients up to Month 6

Phase	Screening Phase [a]	Conditioning Phase LDC	Treatment and Primary Follow-up Phase											
Days	≤28 days Before LDC	D -5, -4, -3	D0 [b]	D1	D3	D7	D10	D14	D21	D28/ M1	M2	M3	M4	M6
Visit window				±1 d	±1 d	±1 d	±1 d	±2 d	±2 d	±4 d	±4 d	±14 d	±14 d	±14 d
Safety assessments (continued)														
Virus tests (CMV, HBV, HCV, HIV)	X	For patients with evidence of prior exposure to HBV or HCV: HBV DNA or HCV RNA at D28/M1, M2, M3, M4, and M6. For all other cases: Test as clinically indicated OR if ALT > 3 × ULN. Patients who are HIV-positive but have a negative viral load at screening must continue to have HIV viral load monitored per institutional guidelines for the duration of the study. Not required for a patients with documented disease progression and/or begins a new systemic anticancer treatment at the prior visit.												
Concomitant medications and procedures	<---------- X ---------->													
AE [i]	<---------- X ---------->													
Survival follow-up [j]												X		X
Study Intervention														
LDC		D -5, -4, -3												
TAK-007 Administration			X [b]											
Disease assessments														
Lymphoma tissue collection	X					(Range of D7-D14)								
Disease assessment per Lugano classification [l]	X									X		X[c]		X[c]
BM biopsy [m]	X									Mandatory for iNHL at the time of CR in imaging (for patients with previous BM involvement)				
BM aspirate [n]	X					(Range of D7-D14)				Recommended for iNHL at the time of CR in imaging (for patients with previous BM involvement) and recommended at relapse (for all patients)				

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Table A 1 Schedule of Events for All Patients up to Month 6

Phase	Screening Phase [a]	Conditioning Phase LDC	Treatment and Primary Follow-up Phase											
Days	≤28 days Before LDC	D -5, -4, -3	D0 [b]	D1	D3	D7	D10	D14	D21	D28/ M1	M2	M3	M4	M6
Visit window				±1 d	±1 d	±1 d	±1 d	±2 d	±2 d	±4 d	±4 d	±14 d	±14 d	±14 d

AE: adverse event; ALT: alanine aminotransferase; BM: bone marrow; CMV: cytomegalovirus; CR: complete response; CT: computed tomography; D: day; ECG: electrocardiogram; ECHO: echocardiogram; ECOG: Eastern Cooperative Oncology Group; EOS: end of study; FFPE: formalin-fixed paraffin embedded; HBV: hepatitis B virus; HCV: hepatitis C virus; HIV: human immunodeficiency virus; iNHL: indolent non-Hodgkin lymphoma; IWG: International Working Group; LBCL: large B-cell lymphoma; LDC: lymphodepleting chemotherapy; M: month; MUGA: multigated acquisition scan; PD: progressive disease; PET: positron emission tomography; RNA: ribonucleic acid; SpO_2: oxygen saturation; ULN: upper limit of normal.

Tests and procedures should be performed on schedule for all visits. Evaluations and procedures identified in the schedule of events may be performed at unscheduled visits, as clinically indicated, at the investigator's discretion in consultation with the sponsor. If a patient is not able to visit the study site, then a telehealth visit should be considered to ensure appropriate oversight and safety monitoring. In addition to this, if the patient is able to visit a local doctor and/or laboratory, then appropriate assessments may be performed at a local laboratory or doctor's office, per investigator discretion.

Day 0 will be defined as the actual day of first TAK-007 administration, hence the reference of all subsequent dates.

After the Day 28 visit, each month will count as 30 days.

[a] The screening visit must occur within 28 days before the day of LDC. Signed informed consent must be obtained before performing any protocol-specific procedure.

[b] All assessments must be completed and evaluated before dosing.

[c] Not required for a patient with documented disease progression and/or begins a new systemic, anticancer therapy at the prior visit.

[d] For women of childbearing potential, a serum pregnancy test will be performed at screening. A urine pregnancy test will be allowed for all other visits. A negative pregnancy test result on Day -5 is necessary before initiation of LDC. If menstrual period is delayed during the study, absence of pregnancy must be confirmed by serum pregnancy test.

[e] Height to be measured at screening only.

[f] Vital signs and SpO_2: Before TAK-007 administration and until the patient is clinically stable, at a minimum, 15, 30, 45, 60 minutes (±5 minutes) and 1.5, 2, and 3 hours (±10 minutes) after start of TAK-007 administration.

[g] Coagulation will only be assessed at screening and as clinically indicated. Immunoglobulins will not be assessed between Day 0 and Day 21.

[h] Hematology samples will be collected before TAK-007 administration and 1 hour (±15 minutes) after start of TAK-007 administration.

[i] Details on monitoring of AEs and period of observation during the primary follow-up phase are summarized in Section 10.3.

[j] Survival will be followed up for patients who receive TAK-007 administration. Survival will be followed-up every 3 months. If a patient misses a scheduled visit, or if the time-point does not align with a scheduled visit, survival status may be collected by methods that include, but are not limited to, telephone, email, mail, or retrieval from online or other databases (eg, Social Security indexes).

[k] A lymphoma biopsy at screening is mandatory unless archived tumor lymphoma tissue (at least 10 unstained FFPE slides) collected after the last therapy is available. Sponsor waiver may be given on a case by case basis. For all patients, Days 7 to 14 biopsies will be optional. Biopsies at relapse are recommended. There should be no clinical contraindication and lymphoma should be easily accessible.

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Table A 1 Schedule of Events for All Patients up to Month 6

Phase	Screening Phase [a]	Conditioning Phase LDC	Treatment and Primary Follow-up Phase											
Days	≤28 days Before LDC	D -5, -4, -3	D0 [b]	D1	D3	D7	D10	D14	D21	D28/ M1	M2	M3	M4	M6
Visit window				±1 d	±1 d	±1 d	±1 d	±2 d	±2 d	±4 d	±4 d	±14 d	±14 d	±14 d

[l] Disease assessment per IWG 2014 guidelines will be performed until PD by imaging, or additional lymphoma therapy except consolidating autologous or allogeneic stem cell transplantation. Assessments will be performed per local guidance.
[m] Baseline BM evaluation is required for all patients at screening except for patients with LBCL with confirmed BM involvement in PET-CT. Archived BM biopsy sample (and assessment) within 42 days before start of lymphodepleting chemotherapy is acceptable if no antitumor therapy was given after BM evaluation. After baseline evaluation, BM will be evaluated in patients with iNHL who had initial BM involvement and in whom, treatment achieved CR in imaging. There should be no clinical contraindication for BM evaluation.
[n] Optional BM aspiration for pharmacokinetics will be performed between Days 7 and 14. Based on the emerging data, collection from additional patients may be waived or made optional. BM aspirate is also recommended at relapse. There should be no clinical contraindication for BM evaluation.

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Table A 2 Schedule of Events for All Patients up to Month 6 (Treatment and Primary Follow-up Phase) – Biomarker Sampling

Procedure	Conditioning Phase	Treatment and Primary Follow-up Phase								
	LDC	TAK-007 Administration	Post-treatment Follow-up							
	D -5 (before LDC Administration)	D0	D1	D3	D7	D10	D14	D21	D28/ M1	M2, M3, M4 and M6 [a]
Visit window	-4d		±1 d	±1 d	±1 d	±1 d	±2 d	±2 d	±4 d	±4 d (M2 only)/ ±14 d
Blood for DNA (CK)		X[b]	X	X	X	X	X	X	X	X
Blood for flow cytometry (CK)	X	X[b]		X	X	X	X	X	X	M2 and M6[c]
Blood (HLA and KIR typing)	X									
Blood for DNA (RCR)	X									M3 and M6
Blood for T/B/NK cells absolute counts	X	X (before TAK-007 administration			X				X	M2, M3 and M6[c]
Blood for immunophenotyping	X				X		X		X	M3 and M6[c]
Plasma for IL-15 and cytokine panel [d]	X	X (before TAK-007 administration)	X		X		X	X	X	M2 and M6[c]
Serum for humoral immunogenicity (anti-HLA and anti-CAR antibody)	X								X	X[c]
Plasma for ctDNA	X	X (before TAK-007 administration)					X		X	M3 and M6[c]
Blood for cellular immunogenicity	X								X	X[c]

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Table A 2 Schedule of Events for All Patients up to Month 6 (Treatment and Primary Follow-up Phase) – Biomarker Sampling

Procedure	Conditioning Phase	Treatment and Primary Follow-up Phase								
	LDC	TAK-007 Administration	Post-treatment Follow-up							
	D -5 (before LDC Administration)	D0	D1	D3	D7	D10	D14	D21	D28/ M1	M2, M3, M4 and M6 [a]
Visit window	-4d		±1 d	±1 d	±1 d	±1 d	±2 d	±2 d	±4 d	±4 d (M2 only)/ ±14 d

CAR: chimeric antigen receptor; CK: cellular kinetics; ctDNA: circulating tumor DNA; D: day; HLA: human leukocyte antigens; IL 15 interleukin-15; KIR: killer immunoglobulin-like receptor; LDC: lymphodepleting chemotherapy; M: month; NK: natural killer; RCR: replication competent retrovirus.
Unless otherwise noted, evaluations during the conditioning and treatment phases must occur before LDC administration or TAK-007 administration. Tests and procedures should be performed on schedule for all visits. If extenuating circumstances prevent a patient from completing a scheduled procedure or assessment within this time, the patient may continue the study only with the written permission of the medical monitor.
After the Day 28 visit, each month will count as 30 days.
[a] For patients who have documented progression at a prior visit and/or begin a new systemic anticancer treatment at a prior visit, only M3 and M6 samples are required where indicated.
[b] Samples will be collected before and after (1 hour ±15 minutes) TAK-007 administration.
[c] Sample is not required if patient has documented progression and/or begin a new systemic anticancer treatment at a prior visit.
[d] Additional samples may be collected during safety assessment if patient develops Grade 3 toxicity.

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Table A 3 Schedule of Events for All Patients from Month 6 onwards

Procedure	Secondary Follow-up M12, M18, and M24 ±2 months	Long Term Follow-up Annually after M24 until 15 years ±3 months
Adverse events [a,b]	<---------- X ---------->	
Second primary malignancy evaluation	X	X
Survival follow-up [c]	X	X
New anticancer therapy	X	X[d]
Blood for DNA (CK) [e]	X	X
Blood for DNA (RCR)	M12 and M24[f]	X[f]
Serum for humoral immunogenicity (anti HLA and anti-CAR antibody)	M12	
Pregnancy test	As clinically indicated until M12	

AE: adverse event; CAR: chimeric antigen receptor; CK: cellular kinetic(s); D: Day; M: Month; PD: progressive disease; RCR: replication competent retrovirus; SAE: serious adverse event; SCT: stem cell transplantation.
Tests and procedures should be performed within the specified windows for all visits. If extenuating circumstances prevent a patient from completing a scheduled procedure or assessment within this time, the patient may continue the study only with the written permission of the medical monitor. If a patient is not able to visit the study site, then a telehealth visit should be considered to ensure appropriate oversight and safety monitoring. In addition to this, if the patient is able to visit a local doctor and/or laboratory, then appropriate assessments may be performed at a local laboratory or doctor's office, per investigator discretion.
Each month will count as 30 days.
[a] During secondary follow-up, all AEs with onset date more than 6 months until 24 months after TAK-007 administration should be reported according to criteria listed in Section 10.3.
[b] During LTFU, all related SAEs, all deaths, and new incidences of Grade ≥3 AEs that are at least possibly related to TAK-007 and/or lymphodepletion and relevant to the potential risks of TAK-007 will be collected (see Section 10.3).
[c] Survival follow-up should be conducted every 6 months in secondary and at least annually during the long-term follow-up phases. If a patient misses a scheduled visit, or if the time-point does not align with a scheduled visit, survival status may be collected by methods that include, but are not limited to, medical records review, telephone, email, mail, or retrieval from online or other databases (eg, Social Security indexes).
[d] New anticancer therapy need not be collected if samples for CK (DNA) are no longer being taken after Month 24.

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Table A 3 Schedule of Events for All Patients from Month 6 onwards

	Secondary Follow-up	Long Term Follow-up
	M12, M18, and M24	Annually after M24 until 15 years
Procedure	±2 months	±3 months

[e] If CAR transgene is detectable at Months 24, 36, or 48, additional samples will be collected within the next 6 months ie, at Months 30, 42, and 54, respectively, for further clonality assessment.
[f] If all post-treatment assays for an individual patient are negative during the first year, collection of the yearly follow-up samples may be discontinued for that individual.

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